

Amputation

Sometimes, if the primary bone cancer or soft tissue sarcoma has spread, the affected limb is amputated. You will have counselling and support before and after surgery.

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What is an amputation?

An amputation is an operation to remove all or part of a limb. This type of surgery is sometimes used to treat [primary bone cancer](#) or [soft tissue sarcoma](#) in an arm or leg.

It is usually possible to remove a cancer in an arm or leg without having to remove the whole limb. This is called [limb-sparing surgery](#).

Rarely, the surgeons may need to remove (amputate) all or part of a limb.

They may suggest this if:

- the cancer has spread from the bone into the surrounding blood vessels
- your movement will be better after an amputation than after limb-sparing surgery
- you get an infection in the bone after limb-sparing surgery that does not respond to treatment
- the cancer comes back in the bone.

After talking to your doctor, family or friends, you may choose to have an amputation instead of limb-sparing surgery.

Preparing for an amputation

Before surgery, your cancer doctor and specialist nurse will explain the operation to you. It is important you fully understand what is involved. You can ask questions about how the surgery might affect you. Your treatment team will give you as much support as possible.

Before the operation, you will meet the rehabilitation team who will help you after the operation. The team includes specialist doctors and nurses, as well as:

- a physiotherapist
- an occupational therapist (OT)
- a prosthetist, who fits an artificial limb (prosthesis) if needed.

They can answer your questions and help with any worries you have about after the operation. They will also talk to you about any changes you need to make to your home. These will make it easier when you go home after surgery.

If you are going to have an amputation, it is important to get psychological support. Some hospitals have a counsellor you can talk to about your feelings and worries before and after the operation.

It may also help to talk to someone who has had the same operation. Your healthcare team or keyworker may be able to arrange this for you. You can also visit our [bone cancer forum](#) to talk with people who have been affected by bone cancer. The [Bone Cancer Research Trust](#) website has videos that might also be helpful.

What to expect after an amputation

When you wake up from the operation, you will usually have a drip (intravenous infusion) to keep you hydrated. This can be removed as soon as you are drinking enough.

Your wound

To start with, the area operated on will be bandaged. You will have a tube in the wound to drain any fluid that builds up. This is removed when fluid has stopped draining. This usually takes a few days.

When the bandages have been removed, your physiotherapist or nurse will fit a compression garment. This helps reduce swelling. You will also be given advice about looking after the scar.

Pain

[Pain](#) can usually be controlled effectively with painkillers. They may be given:

- into a vein (intravenously)
- into the space around the spinal cord (epidural)
- into a muscle (intramuscularly)
- as tablets.

If you are in pain, let your nurses and doctors know as soon as possible. This will help them give you the right amount of painkillers.

Some people have pain that feels like it is coming from the part of the limb that has been amputated. This is called phantom pain or phantom sensation. This pain usually fades over time, but there may be some discomfort in the area for a while after the operation. If you have any phantom pain, let your nurses and cancer doctor know. They can give you drugs to help control it, such as pregabalin or gabapentin.

Getting up

A physiotherapist or occupational therapist (OT) will help you get up. This often happens on the first or second day after the operation. It might be hard at first, and you will need to start slowly.

If you have had an arm amputated, you will be able to get up and move around once your pain is controlled and you have recovered from the anaesthetic.

If you have had a leg removed, the physiotherapist will show you how to move around in bed and how to get into a chair. To start with, you will use a wheelchair.

The OT will arrange a wheelchair for you to use in the hospital. You may need the wheelchair at home while you get used to moving around using crutches or an artificial limb. Before you go home, you will be referred to your local wheelchair service. You will also be given advice about using a wheelchair at home.

Exercises

Your nurses or physiotherapist will teach you breathing and leg exercises. The exercises will help prevent you getting a chest infection and blood clots. Chest infections or blood clots can develop if you are not moving around as much as normal. You can help your recovery by doing the exercises as often as you need to.

You may be given elastic stockings called TED stockings to wear to help prevent a blood clot in your leg. This is called a deep vein thrombosis or DVT. You may also be given injections of anti-clotting medicines to help thin your blood.

Physiotherapy

When you are moving around more, you will go to the hospital gym for physiotherapy. This is a very important part of your recovery. Your physiotherapist will give you exercises to do as the wound heals and you gain strength. They will show you how to keep the muscles around the amputated limb strong and supple. This will make it easier to work an [artificial limb](#) if you have one.

Your physiotherapist will arrange for you to keep having physiotherapy when you go home. This will usually be as an outpatient and can often be near your home.

Rehabilitation

Amputation is a major operation, and rehabilitation can take a long time. It can be hard work and may be frustrating at times.

After the amputation, the occupational therapist (OT) will help you manage day-to-day activities, such as washing, dressing and using the toilet. They can also recommend equipment to help you be as independent as possible.

If you have had an arm amputated, your OT may teach you ways to reduce swelling. They will also teach you how to do some activities with 1 hand.

The OT can also give you advice about driving, work and getting back to social activities. They will help you think about how your amputation may affect your life and look at different ways to manage it.

Going home

Before leaving the hospital, your nurse, physiotherapist and occupational therapist (OT) will talk to you about how you will manage at home. They will help you plan how you manage your daily living activities and decide whether you need any equipment. If you live alone or have stairs, you may need some help at home.

The OT can help you find different ways of doing things, and suggest aids that may help. You may need to make changes to your home and the way you do certain activities. The OT may also refer you to your local social services for further assessment.

This can sometimes delay your discharge from hospital, which may be frustrating.

If you have any worries about going home, talk to the nursing staff about them so they can organise some help.

Driving after amputation

If you have had a limb removed, you must tell your car insurance company and the:

- [DVLA](#) in England, Scotland or Wales
- [DVA](#) in Northern Ireland.

Changes can be made to your car to help you drive. Your OT can tell you more about them, and suggest organisations that can make the changes you need.

Blue Badge scheme

This scheme allows people with mobility problems to park closer to where they want to go. [Gov.uk](#) has more information about the Blue Badge scheme and how to apply. Or you can contact your local council for more information. A healthcare professional, welfare rights adviser or social worker can help with the application.

Artificial limb (prosthesis)

After an amputation, most people can be fitted with an artificial limb called a prosthesis. We have more information about [getting your artificial limb](#) and what to expect.

How to cope with amputation

Amputation can be very distressing and can take a long time to come to terms with.

Feelings and emotions

Losing an arm or a leg can feel like a bereavement. You will need time to grieve for your loss and start coping with the emotional and practical problems this type of surgery can bring.

We have more information about dealing with the [emotional effects of cancer](#).

Body image

You may feel shocked and distressed after the operation. You are used to what your body looks like, and it can be very hard to come to terms with a major change such as an amputation.

Feeling that you look different from other people can affect your confidence and how you think and feel about your body (body image). At first, you may find it hard to see people after your amputation. But you and the people close to you will get used to the way you look. This will help you become more confident about dealing with the reactions of people you do not know.

Some people find it helpful to move around as soon as possible after their operation. But it is important to take the time you need after surgery to adapt and do things in your own time. You may want to take someone with you when you first go out, to give you emotional support. You may find other people do not even notice your amputation, especially if you are wearing an artificial limb.

Some people might find it helpful to talk with a psychologist or counsellor. Your doctor or nurse can arrange this for you.

We have more information to help you [cope with body changes](#).

Family and friends

Your family and friends may also find it hard to manage their feelings about your amputation. You may be anxious about what they will say or think, and if you will be able to cope with their reactions. But most people find their family and friends want to do as much as possible to support them. It can help to [talk about your feelings](#) and any worries you have.

Sex and relationships

Some people may feel unattractive and embarrassed about their body. If you have a partner, you may be worried they will not find you attractive. Meeting a new partner may seem scary. We have more information about [cancer and your sex life](#).

Help is available

You will need time and help to come to terms with your emotions, which are likely to be very strong. The hospital staff will know this and will help you as much as they can. Many people find it helpful to [discuss their feelings](#) with a close friend or someone who is removed from their situation, such as a counsellor. Often, there are counsellors or psychologists in the hospital, and the staff can arrange for you to see them.

Our cancer support specialists can talk to you, and tell you how to contact a counsellor or [local cancer support group](#). You can call them on [0808 808 00 00](#).

There are also different organisations that offer support, such as the [Limbless Association](#).

Date reviewed

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⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

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Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

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